

SBA – Psych

A patient with chronic low back pain and long-term opioid use tells you, "I know I probably should cut down, but I'm just not ready yet." Which of the following responses best reflects the principles of motivational interviewing?

Options:

- A. "It sounds like part of you can see the benefits of cutting down, while another part isn't quite ready yet — can you tell me more about what's holding you back?"
- B. "That's completely understandable — change happens when you're ready, so let's just focus on your pain management for now and revisit this another time."
- C. "A lot of my patients feel that way initially, but once they see how much better they feel after reducing their dose, they usually wish they'd started sooner."
- D. "I hear you, but I do need to let you know that continuing at this dose long-term carries real risks, so it's something we should seriously consider addressing soon."
- E. "What would need to change for you to feel more ready to think about cutting down?"

Answer: A — "It sounds like part of you can see the benefits of cutting down, while another part isn't quite ready yet — can you tell me more about what's holding you back?"

Explanation:

Option A explicitly **reflects the patient's ambivalence** back to them (naming both sides of it) and then uses an **open-ended question** to explore it further — combining reflective listening with evocation, the central engine of Motivational Interviewing.

A (correct): Captures the patient's ambivalence accurately and non-judgementally, then invites further exploration with an open question, without steering the patient toward a particular conclusion.

B – "Let's just focus on pain management for now": This sounds accepting and non-confrontational, but it actually **avoids engaging with the ambivalence at all** rather than exploring it — Motivational interviewing doesn't mean dropping the subject, it means exploring it skilfully; this response is passive rather than evocative.

C – "Once they see how much better they feel, they wish they'd started sooner": This may sound like gentle encouragement rather than direct confrontation, but it is actually **installing the clinician's own conclusion** ("you'll feel better, you'll wish you'd started sooner") rather than drawing out the patient's own reasons for change — this conflicts with Motivational Interviewing's evocation principle, even though it isn't aggressive in tone.

D – "I do need to let you know... it's something we should seriously consider addressing soon": This begins with apparent empathy ("I hear you") but pivots into **clinician-delivered persuasion and an implicit push toward a timeline** — a common real-world blend of MI-sounding language with an underlying directive/advice-giving structure that is not true MI, since it still supplies the argument for change rather than eliciting it from the patient.

E – "What would need to change for you to feel more ready?": A genuinely reasonable, open-ended MI-consistent question in isolation, and could arguably also be correct — but it **skips**

past reflecting the ambivalence first, moving straight to a forward-looking question without first demonstrating that the clinician has heard and understood both sides of the patient's current ambivalence; reflective listening typically precedes this kind of evocative question in well-executed MI, making A the more complete and prototypical example.

Knowledge summary Box - Motivational Interviewing

1. Definition

A collaborative, patient-centred communication style for strengthening a person's own motivation and commitment to change, by exploring and resolving ambivalence, rather than imposing change through direct persuasion, confrontation, or authority.

2. Spirit of MI (underlying philosophy)

- **Collaboration** (not confrontation) — working *with* the patient, not imposing expertise *on* them
- **Evocation** (not education/installation) — drawing out the patient's own motivations, rather than installing motivation from outside
- **Autonomy** (not authority) — respecting the patient's right to make their own choices

3. Core principles/techniques — OARS

- **Open-ended questions** — encouraging elaboration rather than yes/no answers
- **Affirmations** — recognising patient strengths and efforts
- **Reflective listening** — reflecting back what the patient has said, including ambivalence, to deepen exploration
- **Summarising** — periodically pulling together what has been discussed

4. Clinical relevance in pain medicine

- Particularly useful for: opioid tapering/reduction discussions, engagement with exercise/physiotherapy programmes, smoking cessation, weight management, and engagement with multidisciplinary pain management programmes

EMQ – Psychology

Options (A–H):

- A. Motivational interviewing
- B. Cognitive behavioural therapy
- C. Acceptance and commitment therapy
- D. Biofeedback
- E. Graded exposure
- F. Shared decision-making
- G. Psychoeducation
- H. Behavioural activation

Select the single most appropriate approach for each scenario. Each option may be used once, more than once, or not at all.

Scenarios:

1. A clinician reflects a patient's ambivalence about reducing opioid use back to them, without directly advising a course of action.
2. A patient with chronic pain and significant fear of movement is guided through a programme of increasing physical activity
3. A clinician and patient discuss risks and benefits of starting a spinal cord stimulator trial versus continuing conservative management
4. A patient is taught to recognise and challenge unhelpful, catastrophic thoughts about their pain
5. A patient is given information explaining the neurophysiology of chronic pain and central sensitisation, to help them understand why their pain persists despite normal imaging.

Answers:

- 1 - A**
- 2 - E**
- 3 - F**
- 4 - B**
- 5 - G**

Explanation:

1 - A: Reflecting ambivalence and exploring the patient's own reasons for/against change via open questions, without directing them toward a predetermined outcome, is the hallmark approach of **motivational interviewing**.

2 - E: A structured, stepwise exposure programme starting with least-feared movements and progressing gradually is the defining technique of **graded exposure**, used to address fear-avoidance and kinesiophobia in chronic pain.

3 - F: Jointly weighing risks/benefits of a specific treatment option while incorporating patient values and preferences into a final, collaborative decision describes **shared decision-making**.

4 - B: Identifying and challenging unhelpful/catastrophic cognitions alongside behavioural strategies is the defining structure of **cognitive behavioural therapy**.

5 - G: Providing structured, factual information to help a patient understand the underlying mechanism of their condition (e.g. central sensitisation) is the definition of **psychoeducation**.

Acceptance and commitment therapy: Focuses on psychological flexibility — accepting pain-related thoughts and sensations without struggle, clarifying personal values, and committing to value-driven action despite ongoing pain.

Biofeedback: Uses real-time physiological monitoring (e.g. EMG, skin temperature) and operant conditioning to teach conscious control over physiological processes contributing to pain/tension.

Behavioural activation: Involves systematically increasing engagement in meaningful, rewarding activities to counteract avoidance and low mood.